

ADR Identification Basics

Objective: To analyze patient case scenarios and identify potential Adverse Drug Reactions (ADRs) by reviewing symptoms, medication history, and timelines.

ADR Assessment Steps

1. Review the patient's medication history.
2. Identify the signs and symptoms experienced by the patient.
3. Check the timeline between drug administration and onset of symptoms.
4. Assess whether the symptoms are consistent with a possible adverse drug reaction.
5. Consider other possible causes or underlying diseases.
6. Identify the suspected drug responsible for the reaction.
7. Assess the seriousness of the reaction.
8. Document the patient's outcome, including recovery or improvement after withdrawal/change when applicable.

Case Analysis

Case	Drug	Symptoms	Assessment	Suspected drug	Status
Case 1	Paracetamol	Itching and fever	Review the timing and medication history and exclude alternative causes before attributing the symptoms to the drug.	Paracetamol	Potential ADR
Case 2	Drug A / Drug B	Headache and dizziness	Compare the exposure and symptom-onset timelines for both medicines and assess other possible causes before identifying the most likely suspect.	Drug A or Drug B — requires timeline assessment	Potential ADR
Case 3	Drug C	Abdominal pain and vomiting	The symptoms should be assessed against the medication timeline. The case is described as serious and requires appropriate clinical assessment.	Drug C	Potential serious ADR
Case 4	Drug D	Nausea, vomiting and rash	Review the onset after drug exposure and consider alternative explanations. The combination may represent a possible drug-related reaction.	Drug D	Potential ADR
Case 5	Drug E	Low blood sugar, sweating and dizziness	Assess the relationship to drug exposure and document clinical outcome/recovery status. The case is described as serious.	Drug E	Potential serious ADR

Conclusion

ADR identification requires a systematic evaluation of symptoms, medication history, timing of exposure, alternative causes, seriousness, and clinical outcome. Proper identification of the suspected drug supports appropriate pharmacovigilance assessment and documentation.

Note: These case assessments are educational examples. Final clinical causality assessment should consider the complete patient history and relevant clinical information.